

**PATIENT NAME : MR. ANSH H MEHTA****REF. DOCTOR : DR. PREVENTIVE HEALTH CARE****CODE/NAME & ADDRESS : C000151498**

CHC MGM MALAR (HLM)

NO.111 OLD NO.52, 1ST MAIN ROAD, GANDHI

NAGAR ADYAR, CHENNAI

CHENNAI 600020

ACCESSION NO : 0398ZI001309**PATIENT ID : ANSHM070203398**

CLNT.PATIENT ID: 90001404588

ABHA NO :**AGE/SEX : 23 Years Male****DRAWN :****RECEIVED : 12/09/2026 14:53:58****REPORTED : 12/09/2026 18:52:26****CLINICAL INFORMATION :**

UHID : 90001404588

Encounter No :

OP 9032609124237

Test Report Status Final**Results****Biological Reference Interval Units****SPECIALISED CHEMISTRY - HORMONE****TSH 3RD GENERATION ULTRASENSITIVE, SERUM****TSH (ULTRASENSITIVE)****4.010****0.270 - 4.200****μIU/mL**

METHOD : ELECTROCHEMILUMINESCENCE

Interpretation(s)

TSH 3RD GENERATION ULTRASENSITIVE, SERUM-**TSH stands for thyroid stimulating hormone.** This hormone stimulates the Thyroid gland to make thyroid hormones that regulate the way our body uses energy. These also play an important role in regulating weight, temperature, muscle strength, and even your mood. TSH is made in a gland in the brain called the pituitary. When thyroid levels in our body are low, the pituitary gland makes more TSH. When thyroid levels are high, the pituitary gland makes less TSH. TSH levels that are too high or too low can indicate that thyroid is not working correctly.

There is a circadian rhythm of TSH secretion, with peak values at the onset of sleep and nadir concentrations during the afternoon hours. Peak and nadir concentrations differ by approximately +/- 50%. The effect on circulating T4 and T3 concentrations is not significant because of the large size of the extrathyroidal T4 pool.

In healthy subjects there is no significant impact of body weight, physical training, body habitus, posture, immobilization, mild to moderate exercise, or ambulatory status on thyroid function, and no significant geographic environmental variation. Nutrition also has a minimal impact except for variation in iodine intake. Subthreshold concentrations of iodine intake are associated with increased TSH secretion, goiter, increased thyroid iodine uptake, decreased T4 production, an increased T3/T4 secretion ratio, and an increased ratio of circulating T3/T4 concentrations. Excessive iodine intake can block thyroid hormone biosynthesis by inhibiting the enzymes involved in the biosynthetic process, resulting in reduced T4 secretion, increased TSH concentrations, goiter, and hypothyroidism if the iodine excess is chronic.

High TSH levels can mean your thyroid is not making enough thyroid hormones, a condition called hypothyroidism. **Low TSH levels** can mean your thyroid is making too much of the hormones, a condition called hyperthyroidism. A TSH test does not explain why TSH levels are too high or too low.

In cases of Subclinical hypothyroidism, a single test can be misleading, so a second test is usually done 2 or 3 months later. In both tests, the blood is taken at the same time of day because TSH levels can fluctuate over the course of 24 hours. Subclinical hypothyroidism is diagnosed when both TSH readings are high but the thyroid hormone thyroxine is still within the normal range.

Being severely overweight and certain medications can also increase TSH. TSH levels are likely to fluctuate more during pregnancy.

TSH values may be transiently altered because of Non thyroidal Illness like severe infections, liver disease, renal failure, heart failure, severe burns, trauma, surgery etc.

TSH levels that are slightly or only moderately elevated do not necessarily need to be treated. Some people who have high TSH levels never even develop symptoms.

It is also very common for TSH levels to return to normal in children and teenagers.

REF: 1. TIETZ Fundamentals of Clinical chemistry 2 Guidelines of the American Thyroid association during pregnancy and Postpartum, 2011.

TSH in pregnancy

There's reduction in both the lower and the upper limit of maternal TSH relative to the non-pregnant TSH reference range. This is because of elevated levels of serum hCG that directly stimulates the TSH receptor, thereby increasing thyroid hormone production. The largest decrease in serum TSH is observed during the first trimester.

Thereafter, serum TSH and its reference range gradually increases in the second and third trimesters, but nonetheless remains lower than in non-pregnant women.

****End Of Report******Please visit www.agilusdiagnostics.com for related Test Information for this accession****Dr. Gayathri K**
Consultant Pathologist

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**ULR No. 775000019853644-0323**

PATIENT NAME : MR. ANSH H MEHTA**REF. DOCTOR : DR. PREVENTIVE HEALTH CARE****CODE/NAME & ADDRESS :** C000151498

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Test Report Status **Final****Results****Biological Reference Interval Units****HAEMATOLOGY - CBC****CBC WITH ESR (COMPLETE BLOOD COUNT WITH ESR),EDTA****BLOOD COUNTS, EDTA WHOLE BLOOD**

HEMOGLOBIN (HB)	12.7 Low	13.0 - 17.0	g/dL
RED BLOOD CELL (RBC) COUNT	5.70 High	4.5 - 5.5	mil/ μ L
WHITE BLOOD CELL (WBC) COUNT	5.45	4 - 10	thou/ μ L
PLATELET COUNT	359	150 - 410	thou/ μ L

RBC AND PLATELET INDICES

HEMATOCRIT (PCV)	40.4	40.0 - 50.0	%
MEAN CORPUSCULAR VOLUME (MCV)	71.0 Low	83.0 - 101.0	fL
MEAN CORPUSCULAR HEMOGLOBIN (MCH)	22.3 Low	27.0 - 32.0	pg
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION(MCHC)	31.4 Low	31.5 - 34.5	g/dL
RED CELL DISTRIBUTION WIDTH (RDW)	18.8 High	11.6 - 14.0	%
MEAN PLATELET VOLUME (MPV)	9.7	6.8 - 10.9	fL

WBC DIFFERENTIAL COUNT

NEUTROPHILS	49	40 - 80	%
LYMPHOCYTES	43 High	20 - 40	%
MONOCYTES	5	2.0 - 10.0	%
EOSINOPHILS	3	1 - 6	%
BASOPHILS	0	0 - 2	%
ABSOLUTE NEUTROPHIL COUNT	2.67	2 - 7	thou/ μ L
ABSOLUTE LYMPHOCYTE COUNT	2.34	1.0 - 3.0	thou/ μ L
ABSOLUTE MONOCYTE COUNT	0.27	0.20 - 1.00	thou/ μ L
ABSOLUTE EOSINOPHIL COUNT	0.16	0.02 - 0.50	thou/ μ L
ABSOLUTE BASOPHIL COUNT	0.00	0.0 - 0.10	thou/ μ L

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ULR No.775000019853644-0398

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OP 9032609124237

Test Report Status Final**Results****Biological Reference Interval Units****ERYTHROCYTE SEDIMENTATION RATE (ESR),WHOLE BLOOD**

E.S.R

03

0 - 10

mm at 1 hr

Interpretation(s)

RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.

WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease. (Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients A.-P. Yang, et al. International Immunopharmacology 84 (2020) 106504)

This ratio element is a calculated parameter and out of NABL scope.

ERYTHROCYTE SEDIMENTATION RATE (ESR),WHOLE BLOOD-TEST DESCRIPTION :- Erythrocyte sedimentation rate (ESR) is a test that indirectly measures the degree of inflammation present in the body. The test actually measures the rate of fall (sedimentation) of erythrocytes in a sample of blood that has been placed into a tall, thin, vertical tube. Results are reported as the millimetres of clear fluid (plasma) that are present at the top portion of the tube after one hour. Nowadays fully automated instruments are available to measure ESR.

- ESR is not diagnostic it is a non-specific test that may be elevated in a number of different conditions. It provides general information about the presence of an inflammatory condition.CRP is superior to ESR because it is more sensitive and reflects a more rapid change.

TEST INTERPRETATION : Increase in: Infections, Vasculitis, Inflammatory arthritis, Renal disease, Anemia, Malignancies and plasma cell dyscrasias, Acute allergy Tissue injury, Pregnancy, Estrogen medication, Aging.

Finding a very accelerated ESR(>100 mm/hour) in patients with ill-defined symptoms directs the physician to search for a systemic disease (Paraproteinemias, Disseminated malignancies, connective tissue disease, severe infections such as bacterial endocarditis).

In pregnancy BRI in first trimester is 0-48 mm/hr(62 if anemic) and in second trimester (0-70 mm /hr(95 if anemic). ESR returns to normal 4th week post partum.

Decreased in: Polycythemia vera, Sickle cell anemia

LIMITATIONS : False elevated ESR : Increased fibrinogen, Drugs(Vitamin A, Dextran etc), Hypercholesterolemia

False Decreased : Poikilocytosis,(SickleCells,spherocytes),Microcytosis, Low fibrinogen, Very high WBC counts, Drugs(Quinine, salicylates)

REFERENCE : Nathan and Oski's Haematology of Infancy and Childhood, 5th edition 2. Paediatric reference intervals. AACCPress, 7th edition. Edited by S. Soldin 3. The reference for the adult reference range is "Practical Haematology by Dacie and Lewis,10th edition.

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ULR No.775000019853644-0398

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ACCESSION NO : **0398ZI001309**

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CLINICAL INFORMATION :

UHID : 90001404588

Encounter No :

OP 9032609124237

Test Report Status **Final****Results****Biological Reference Interval** **Units****HAEMATOLOGY****GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD**

HBA1C	4.3	Non-diabetic: < 5.7 Pre-diabetics: 5.7 - 6.4 Diabetics: > or = 6.5 ADA Target: 7.0 Action suggested: > 8.0	%
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ESTIMATED AVERAGE GLUCOSE(EAG)	76.7	< 116	mg/dL
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Interpretation(s)

GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD-Glycosylated hemoglobin (GHb) has been firmly established as an index of long-term blood glucose concentrations and as a measure of the risk for the development of complications in patients with diabetes mellitus. Formation of GHb is essentially irreversible, and the concentration in the blood depends on both the life span of the red blood cell (average 120 days) and the blood glucose concentration. Because the rate of formation of GHb is directly proportional to the concentration of glucose in the blood, the GHb concentration represents the integrated values for glucose over the preceding 6-8 weeks. Any condition that alters the life span of the red blood cells has the potential to alter the GHb level. Samples from patients with hemolytic anemias will exhibit decreased glycated hemoglobin values due to the shortened life span of the red cells. This effect will depend upon the severity of the anemia. Samples from patients with polycythemia or post-splenectomy may exhibit increased glycated hemoglobin values due to a somewhat longer life span of the red cells. Glycosylated hemoglobins results from patients with HbSS, HbCC, and HbSC and HbD must be interpreted with caution, given the pathological processes, including anemia, increased red cell turnover, transfusion requirements, that adversely impact HbA1c as a marker of long-term glycemic control. In these conditions, alternative forms of testing such as glycated serum protein (fructosamine) should be considered.

References

1. Tietz Textbook of Clinical Chemistry and Molecular Diagnostics, edited by Carl A Burtis, Edward R.Ashwood, David E Bruns, 4th Edition, Elsevier publication, 2006, 879-884.
2. Forsham PH. Diabetes Mellitus:A rational plan for management. Postgrad Med 1982, 71,139-154.
3. Mayer TK, Freedman ZR: Protein glycosylation in Diabetes Mellitus: A review of laboratory measurements and their clinical utility. Clin Chim Acta 1983, 127, 147-184.

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Test Report Status Final Results Biological Reference Interval Units**BIOCHEMISTRY****GLUCOSE FASTING (FBS), FLUORIDE PLASMA**FBS-FASTING BLOOD SUGAR(GLUCOSE) 91 (Normal <100, Impaired fasting glucose: 100 to 125, Diabetes mellitus: ≥ 126 (on more than 1 occasion) (ADA guidelines 2024))**CREATININE EGFR- EPI, SERUM**CREATININE 0.91 0.90 - 1.30 mg/dL
AGE 23 years
GLOMERULAR FILTRATION RATE (MALE) 121.45 mL/min/1.73m²**Interpretation(s)**

eGFR (ml/min/1.73 sq.meters)	Interpretation
≥ 90	Normal
60 - 89	Mild Decrease in GFR
30 - 59	Moderate Decrease in GFR
15 - 29	Severe Decrease in GFR
<15	End stage renal failure

- Kidney disease outcomes quality initiative (KDOQI) guidelines state that estimation of GFR is the best overall indices of the Kidney function.
- It gives a rough measure of number of functioning nephrons. Reduction in GFR implies progression of underlying disease.
- The GFR is a calculation based on serum creatinine test.
- Creatinine is mainly derived from the metabolism of creatine in muscle, and its generation is proportional to the total muscle mass. As a result, mean creatinine generation is higher in men than in women, in younger than in older individuals, and in blacks than in whites.
- Creatinine is filtered from the blood by the kidneys and excreted into urine at a relatively steady rate.
- When kidney function is compromised, excretion of creatinine decreases with a consequent increase in blood creatinine levels. With the creatinine test, a reasonable estimate of the actual GFR can be determined.
- This equation takes into account several factors that impact creatinine production, including age, gender, and race.
- CKD EPI (Chronic kidney disease epidemiology collaboration) equation performed better than MDRD equation especially when GFR is high (>60 ml/min per 1.73m²). This formula has less bias and greater accuracy which helps in early diagnosis and also reduces the rate of false

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Test Report Status Final**Results****Biological Reference Interval Units**

positive diagnosis of CKD.

References:

National Kidney Foundation (NKF) and the American Society of Nephrology (ASN).

Estimated GFR Calculated Using the CKD-EPI equation <<https://testguide.labmed.uw.edu/guideline/egfr>>

Ghuman JK, et al. Impact of Removing Race Variable on CKD Classification Using the Creatinine-Based 2021 CKD-EPI Equation. Kidney Med 2022, 4:100471. 35756325

Harrison Principle of Internal Medicine, 21st ed. pg 62 and 334

ASPARTATE AMINOTRANSFERASE (AST/SGOT),SERUMASPARTATE AMINOTRANSFERASE
(AST/SGOT)

17

0 - 40

U/L

ALANINE AMINOTRANSFERASE (ALT/SGPT), SERUM

ALANINE AMINOTRANSFERASE (ALT/SGPT)

13

0 - 41

U/L

TOTAL PROTEIN, SERUM

TOTAL PROTEIN

8.4 High

6.4 - 8.3

g/dL

Interpretation(s)**GLUCOSE FASTING (FBS), FLUORIDE PLASMA-TEST DESCRIPTION**

Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and so that no glucose is excreted in the urine.

Increased in: Diabetes mellitus, Cushing's syndrome (10 - 15%), chronic pancreatitis (30%). Drugs: corticosteroids, phenytoin, estrogen, thiazides.**Decreased in:** Pancreatic islet cell disease with increased insulin, insulinoma, adrenocortical insufficiency, hypopituitarism, diffuse liver disease, malignancy (adrenocortical, stomach, fibrosarcoma), infant of a diabetic mother, enzyme deficiency diseases (e.g. galactosemia), Drugs: insulin, ethanol, propranolol, sulfonylureas, tolbutamide, and other oral hypoglycemic agents.**NOTE:** While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values), there is wide fluctuation within individuals. Thus, glycosylated hemoglobin (HbA1c) levels are favored to monitor glycemic control.

High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment, Renal Glycosuria, Glycaemic index & response to food consumed, Alimentary Hypoglycemia, Increased insulin response & sensitivity etc.

Glucose fasting, plasma as per ADA Guideline

NORMAL RANGE: <100 mg/dl

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PRE-DIABETIC RANGE: 100-125 mg/dl (Impaired Fasting Glucose)

DIABETIC RANGE: = 126 mg/dl

ASPARTATE AMINOTRANSFERASE (AST/SGOT),SERUM-Aminotransferase (AST) is an enzyme found in various parts of the body .AST is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health. AST levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. AST levels may also increase after a heart attack or strenuous activity.

ALANINE AMINOTRANSFERASE (ALT/SGPT), SERUM-ALT test measures the amount of this enzyme in the blood.ALT is found mainly in the liver, but also in smaller amounts in the kidneys,heart,muscles and pancreas.It is commonly measured as a part of a diagnostic evaluation of hepatocellular injury to determine liver health.AST levels increase during acute hepatitis, sometimes due to a viral infection, ischemia to the liver, chronic hepatitis,obstruction of bile ducts,cirrhosis.

TOTAL PROTEIN, SERUM-is a biochemical test for measuring the total amount of protein in serum.Protein in the plasma is made up of albumin and globulin.

Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma,Waldenstroms disease.

Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage),Burns,Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic syndrome,Protein-losing enteropathy etc.



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CHOLESTEROL, TOTAL	144	Desirable <200 Borderline: 200-239 High >240	mg/dL
TRIGLYCERIDES	129	Normal <150 Borderline high 150-199 High 200-499 Very High >= 500	mg/dL
HDL CHOLESTEROL	24 Low	< 40 Low >=60 High	mg/dL
LDL CHOLESTEROL, DIRECT	96	< 100 Optimal 100 - 129 Near or above optimal 130 - 159 Borderline High 160 - 189 High >= 190 Very High	mg/dL
NON HDL CHOLESTEROL	120	Optimal <130 Desirable 130-159 Borderline High 160-189 High 190-219 Very high >=220	mg/dL
VERY LOW DENSITY LIPOPROTEIN CHOL/HDL RATIO	25.8 6.0 High	< /= 30 Optimal < 3.5 Desirable 3.5 - 5.0 High Risk > 5.0	mg/dL
LDL/HDL RATIO	4.0 High	0.5-3 Desirable/Low risk 3.1-6 Borderline/Moderate risk >6.0 High Risk	

Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non

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Test Report Status **Final****Results****Biological Reference Interval** **Units**

HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A.CAD with > 1 feature of high risk group
	B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease
Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

Elakkiya

Dr.Elakkiya Ilavarasan
Consultant Pathologist & LAB
Head

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View Report

PERFORMED AT :**Agilus Diagnostics Ltd**

B- Block, 8th Floor, #52, first main road, Adyar,

Chennai, 600020

Tamil Nadu, India



ULR No.775000019853644-0398

PATIENT NAME : MR. ANSH H MEHTA**REF. DOCTOR : DR. PREVENTIVE HEALTH CARE****CODE/NAME & ADDRESS : C000151498**CHC MGM MALAR (HLM)
NO.111 OLD NO.52, 1ST MAIN ROAD, GANDHI
NAGAR ADYAR, CHENNAI
CHENNAI 600020**ACCESSION NO : 0398ZI001309****PATIENT ID : ANSHM070203398****CLNT.PATIENT ID: 90001404588****ABHA NO :****AGE/SEX : 23 Years Male****DRAWN :****RECEIVED : 12/09/2026 14:53:58****REPORTED : 12/09/2026 21:39:18****CLINICAL INFORMATION :****UHID : 90001404588****Encounter No :****OP 9032609124237****Test Report Status Final****Results****Biological Reference Interval Units****CLINICAL PATH - URINALYSIS****URINE ROUTINE / URINE ANALYSIS****PHYSICAL EXAMINATION, URINE**

COLOR	PALE YELLOW
APPEARANCE	CLEAR

CHEMICAL EXAMINATION, URINE

PH	5.5	4.5 - 7.5
SPECIFIC GRAVITY	1.015	1.005 - 1.030
PROTEIN	NOT DETECTED	NOT DETECTED
GLUCOSE	NOT DETECTED	NOT DETECTED
KETONES	NOT DETECTED	NOT DETECTED
BLOOD	NOT DETECTED	NOT DETECTED
BILIRUBIN	NOT DETECTED	NOT DETECTED
UROBILINOGEN	NORMAL	NORMAL
NITRITE	NOT DETECTED	NOT DETECTED
LEUKOCYTE ESTERASE	NOT DETECTED	NOT DETECTED

MICROSCOPIC EXAMINATION, URINE

RED BLOOD CELLS	NOT DETECTED	0 - 2	/HPF
PUS CELL (WBCS)	2-3	0-5	/HPF
EPITHELIAL CELLS	3-5	0-5	/HPF
CASTS	NOT DETECTED		
CRYSTALS	NOT DETECTED		
BACTERIA	NOT DETECTED	NOT DETECTED	
YEAST	NOT DETECTED	NOT DETECTED	

Dr.Elakkiya Ilavarasan
Consultant Pathologist & LAB
Head

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Chennai, 600020
Tamil Nadu, India**ULR No.775000019853644-0398**

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CHC MGM MALAR (HLM)

NO.111 OLD NO.52, 1ST MAIN ROAD, GANDHI

NAGAR ADYAR, CHENNAI

CHENNAI 600020

ACCESSION NO : 0398ZI001309**PATIENT ID : ANSHM070203398**

CLNT.PATIENT ID: 90001404588

ABHA NO :**AGE/SEX : 23 Years Male****DRAWN :****RECEIVED : 12/09/2026 14:53:58****REPORTED : 12/09/2026 21:39:18****CLINICAL INFORMATION :**

UHID : 90001404588

Encounter No :

OP 9032609124237

Test Report Status Final**Results****Biological Reference Interval Units****Interpretation(s)**

The following table describes the probable conditions, in which the analytes are present in urine

Presence of	Conditions
Proteins	Inflammation or immune illnesses
Pus (White Blood Cells)	Urinary tract infection, urinary tract or kidney stone, tumors or any kind of kidney impairment
Glucose	Diabetes or kidney disease
Ketones	Diabetic ketoacidosis (DKA), starvation or thirst
Urobilinogen	Liver disease such as hepatitis or cirrhosis
Blood	Renal or genital disorders/trauma
Bilirubin	Liver disease
Erythrocytes	Urological diseases (e.g. kidney and bladder cancer, urolithiasis), urinary tract infection and glomerular diseases
Leukocytes	Urinary tract infection, glomerulonephritis, interstitial nephritis either acute or chronic, polycystic kidney disease, urolithiasis, contamination by genital secretions
Epithelial cells	Urolithiasis, bladder carcinoma or hydronephrosis, ureteric stents or bladder catheters for prolonged periods of time
Granular Casts	Low intratubular pH, high urine osmolality and sodium concentration, interaction with Bence-Jones protein
Hyaline casts	Physical stress, fever, dehydration, acute congestive heart failure, renal diseases
Calcium oxalate	Metabolic stone disease, primary or secondary hyperoxaluria, intravenous infusion of large doses of vitamin C, the use of vasodilator naftidrofuryl oxalate or the gastrointestinal lipase inhibitor orlistat, ingestion of ethylene glycol or of star fruit (Averrhoa carambola) or its juice
Uric acid	arthritis
Bacteria	Urinary infection when present in significant numbers & with pus cells.
Trichomonas vaginalis	Vaginitis, cervicitis or salpingitis

Reference:

Dr. Elakkiya Ilavarasan
Consultant Pathologist & LAB
Head

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ULR No.775000019853644-0398

PATIENT NAME : MR. ANSH H MEHTA

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UHID : 90001404588

Encounter No :

OP 9032609124237

Test Report Status **Final**

Results

Biological Reference Interval Units

Henrys Clinical Diagnostics and Management by Laboratory Methods 21st edition (pg 410).

****End Of Report****

Please visit www.agilusdiagnostics.com for related Test Information for this accession

Elakkiya

Dr.Elakkiya Ilavarasan
Consultant Pathologist & LAB
Head

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Chennai, 600020

Tamil Nadu, India



ULR No.775000019853644-0398

Department of Preventive Health Checkup

MEDICAL SUMMARY

PATIENT NAME

: MR/MS Ansh

AGE / SEX

: 23 Years / MALE/FEMALE

UHID

DATE

: 12/9/2026

HEALTH CHECK-UP

: **MGM PACKAGE**

EXAMINED BY

: DR MEHER PRASAD Y D

CHIEF COMPLAINTS & HISTORY:

Health Checkup

patient here for general health checkup.

PAST HISTORY:

MEDICAL : none.

SURGICAL : Biopsy for nerve sebaceous done (2017).

MEDICATIONS:

None

PERSONAL HISTORY:

Marital status	: single.	No. Of children	: -
Smoking	: none.	Diet	: Vegetarian.
Alcohol	: well maintained.	Sleep	: 6-7 hrs.
Appetite	: occasionally		
Physical Activity	: does gym.		

DRUG/ FOOD ALLERGIES: none.

FAMILY HISTORY:

Father	: none	Sibling	: -
Mother	: none	Others	: -

PHYSICAL EXAMINATION

GENERAL:

Build : Ht: 180 cm BMI: 22.5
wt: 73 kg
Anaemia : normal.
Icterus : Anicteric Sclera.
Cyanosis :
Clubbing :
Skin : } Normal.
Edema :
Glands :
HEENT :

CARDIOVASCULAR SYSTEM:

Heart Rate : 82/ Min
Rhythm : Regular
BP Sitting : 110/80 Mm of Hg
Chest Shape :
Heart Sounds : S1 & S2 + ✓
Thrills :
Murmurs : No Abnormality.
Waist - 78cms
Hip - 82cms

RESPIRATORY SYSTEM:

Rate & Type : / Min Normal
Breath Sounds : NVBS, BLAET. Chest Wall : Normal

ABDOMEN:

Appearance :
Palpation : } Normal,
Bowel Sounds :
Fluid : } Normal Active Bowel sounds.

CENTRAL NERVOUS SYSTEM:

Cranial Nerves :
Sensory System : } Intact
Motor System :
Reflexes :
Pupil & Fundus :

MUSCULOSKELETAL SYSTEM:

Normal

23/m

NAME: Mr. Ansh H mehta	AGE:	SEX:	DATE: 12/9/20
------------------------	------	------	---------------

OPHTHALMIC EVALUATION CHART

VISUAL ACUITY	RIGHT EYE	LEFT EYE
UNAIDED DISTANCE NEAR	6/24 N6	6/24 N6
WITH GLASS DISTANCE NEAR	6/6 N6	6/6 N8
SPECTACLE CORRECTION	2.25 / 1.25 N6	3.0 / 1.50 N6
AR - 2.25 / 1.25 +1.0 - 3.25 / 1.50 +1.75	N6	N6
COLOUR VISION	N	N
MICROSCOPIC EXAMINATION	none	none
IMPRESSION FUNDUS	wnl	wnl
ADVICE	Ve given	
WITH REGARDS	Cont same glar Re ap opture 6/6	

34 m.c. BS

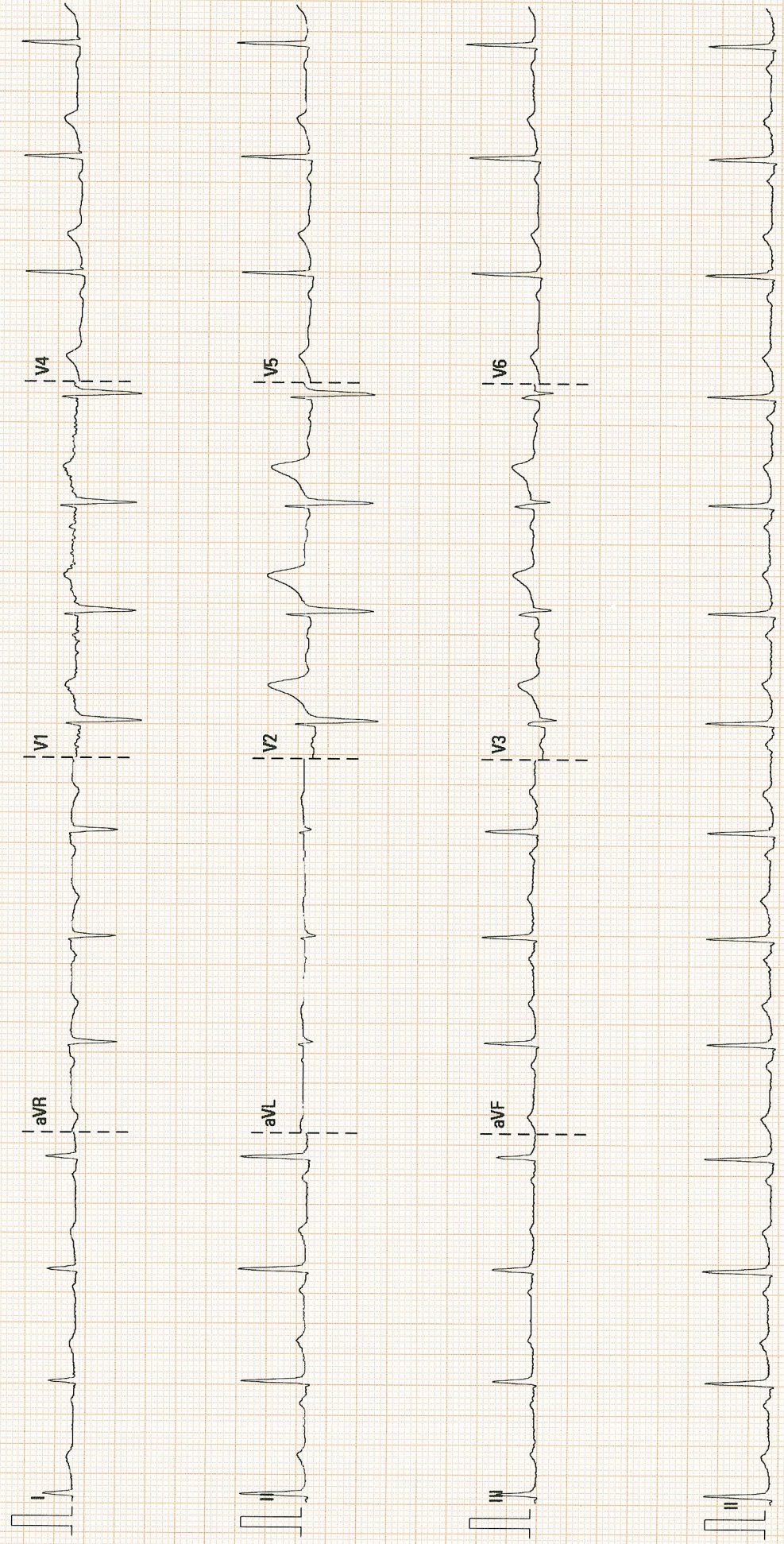
DR. RADHI MALAR ANAND
MBBS/MS, OPHTHALMOLOGIST, FPO(SN)
CONSULTANT OPHTHALMOLOGIST
MGM HEALTHCARE - MALAR ADYAR
REG NO : 73489

12-09-2026 09:56:35 AM

ID: LTI
Name: MR. ANSH H, MEHTA
Age: 23 Years
Gender: Male

Vent. Rate	80 bpm	Sinus rhythm
PR Interval	148 ms	
QRS Duration	82 ms	Normal ECG
QT/QTc Interval	370/405 ms	
P/QRS/T Axes	63/66/58 deg	
QTc/Hodges		

Unconfirmed Diagnosis



Patient Name	ANSH H MEHTA MR	Patient ID	MHC
Age/Sex	23Yr/Male	Study Date	12 Sep 2026
Referral Dr			

CHEST X-RAY (PA VIEW)

The heart size and configuration are within normal limits. The aortic arch is normal.
The lung fields show normal broncho-vascular markings.
Both the pulmonary hila are normal in size.
The costophrenic and cardiophrenic recesses and the domes of diaphragm are normal.
The bones and soft tissues of the chest wall show no abnormality.

IMPRESSION :

Normal study.


Dr. Prashant Moorthy MDRD.,
Consultant Radiologist
Reg : 100074